Patient Safety Incident Reporting Form

FACILITY: New Somerset Hospital

Section A (notification) - to be completed by manager of section where incident took place. Submit section A and B to next level for notification for SAC 1 incidents

Section B(Statement by staff, patient or significant other)– to be completed by staff, patients or significant other that were directly involved while the incident took place

Section C(investigation) - to be completed by investigator(s) of the incident, in most cases this would be the manager(s) of section where the incident took place

SECTION A - Notification

1. Type of patient safety incident (PSI): Mark with an X		
No harm	Near miss	Harmful (Adverse Event)

2. Patient information		3. Staff involved		
Patient name and surname		Name and surname	Contact detail	Department
Patient file number				
Location (department/ward)				
Age				
Gender				
Final diagnosis				

4. Date of PSI	dd/mm/yyyy	5. Time of PSI	

6. SAC rating: Mark with an X	1	2	3	7. Date reported to next level if SAC = 1	dd/mm/yyyy	8. No of days to report PSI with SAC = 1	

9. Method of detecting PSI: Mark with an X	Reported by health professional	Research studies	Surveys on patient experience of care	Inpatient medical review	Review of record on follow-up	External sources			Safety walk rounds	Focused teams	Use of data
						Complaints	Media	Public			

10. Short description of Patient Safety Incident (detailed information available under section B as reported by staff)

11. Immediate resulting action taken to minimise harm

12. Short description of initial disclosure

Compiled by:	Designation:	Signature:	Date:

SECTION B- Statement by staff

1. Statement by staff: (Add sections for additional statements and information as needed)
Statement 1:

Compiled by:	Designation:	Signature:	Date:

SECTION C - Investigation

1. Category according to type – mark appropriate one with an X

1.Clinical admini-stration	2. Clinical process/ procedure	3. Health care-associated infections	4. Medication / IV fluids	5. Blood and blood products	6. Medical device
Medical procedure performed without valid consent	Not performed when indicated	Central line associated Blood Stream Infection	Wrong dispensing	Acute transfusion reactions	Lack of availability
	Performed on wrong patient	Peripheral Line Infection	Omitted medicine or dose	Delayed transfusion reactions/ events (including Transfusion Transmitted Infections)	Failure / malfunction
	Wrong process/ procedure/ treatment performed	Surgical site	Medicine not available	Errors- wrong blood/ blood products	**8. Patient Accidents**
	Retention of foreign object	Hospital acquired pneumonia	Adverse drug reaction	**7. Behaviour**	Falls
	Pressure ulcers acquired during admission	Ventilator associated pneumonia	Wrong medicine	Suicide	**9. Infrastructure/ Buildings/ Fixtures**
	Performed on wrong body part/ site/side	Catheter associated urinary tract infection	Wrong patient	Attempted suicide	Non-Existent/ inadequate
	Maternal death	Communicable diseases	Wrong frequency	Self inflicted injury	Damaged/ faulty/ worn
	Neonatal death		Wrong route	Sexual assault by staff member	**10. Other**
	Fresh still born		Prescription error	Sexual assault by fellow patient or visitor	Any other incident that does not fit into categories 1 to 9
			Wrong dose/ strength administered	Physical assault by staff member	
				Physical assault by fellow patient or visitor	
				Exploitation, abuse, neglect or degrading treatment by fellow patient or visitor	
				Exploitation, abuse, neglect or degrading treatment by staff member	
				Wandering/ abscond	
				Refusal of treatment	

2. Framework for root cause analysis and implementation of action plans

a. Contributing factors – Mark with an X

1. Staff	Cognitive	Performance	Behaviour	Communication	Patho-Physiological/ Disease		
2. Patient	Cognitive	Behaviour	Communication	Patho-Physiological/ disease	Emotional	Social	
3. Work/environment	Physical environmental / infrastructure	Remote/ long distance from service	Equipment	Consumables	Environmental risk	Current Code/ specifications/ regulations	Security/safety
4. Organisational/service	Protocols/policies/procedures	Processes	Organisational management/decisions/culture	Organisation of teams	Staff establishment		
5. External	Natural Environment	Equipment, Products,	Services, systems and policies				
6. Other							

b. Root cause analysis				
Contributing factor	**Describe the factor that contributed to the event**	**Describe the action plan to rectified the identified problem**	**Person responsible for implementing the action plan**	**Date for implementation**

3. Findings and recommendations by Patient Safety Committee

4. Conclusion			
Type of behaviour according to Just Culture: mark with a X	**No error**	**Human error**	

5. Summary of final disclosure to patient/family

6. Date of closure of PSI case	dd/mm/yyy	**7. No days to close PSI case**		**8. Type of closure: mark with an X**	**PSI case concluded**	**Litigation**	**Referred to labour relations**

9. Patient outcome according to degree of harm: Mark with an X	None	Mild	Moderate	Severe	Death

10. Organisational outcome: Mark with an X	Property damage	Increase in required resource allocation for patient	Media attention	Formal complaint	Damaged reputation	Legal ramifications	Other

Compiled by:	Designation:	Signature:	Date: